

PAIN, SENSITISATION AND LOADING

When the emergency response centre evaluates the information it receives from the guardians, it tries to assess how dangerous the situation is. After sustaining an injury, and in situations where you are unsure of the consequences of an injury, the response centre can tell the guardians to be extra alert. Consider this like the burglar alarm in your house going off when you visit the bathroom in the middle of the night – the alert system has become more sensitive.

Imagine having previously injured your elbow. The injury kept you away from climbing for a long period of time, and you have finally recuperated and are about to get back into climbing again. Are you going to be extra sensitive with how your elbow feels during and after climbing? Most of us probably would be, and this heightened alertness makes our response centre more sensitive to load, among other things. This means that it will take less load for the guardians to send information to the response centre and the siren will sound even though there's no need for protection. You're not even close to putting enough stress on your elbow to get injured, but it still hurts. Breaking this circle requires you to expose yourself to stress and learn how to interpret the pain response.

If you are sure that you're not loading your body through climbing and training to the extent of injury, you will be better equipped to answer YES to the three questions earlier in the chapter. An effective tool in this regard can be a training log:

DAY	1	2	3	4	5	6
What	Bouldering 60 minutes	Lead 45 minutes	Bouldering 30 minutes	Lead 60 minutes	Bouldering 60 minutes	Bouldering 45 minutes
Intensity	Low	Low	Moderate	Low	Low	Moderate
Pain 0–10	Before: 2 After: 4	Before: 2 After: 3	Before: 2 After: 4	Before: 2 After: 2	Before: 1 After: 2	Before: 1 After: 3

With a training log, you can see what and how hard you have trained in relation to the pain response. It's common to rate the pain response on a scale from 0 to 10, where 0 equals no pain and 10 is the worst possible pain you can imagine. An increase in load usually leads to a higher response, but if the pain goes back to its starting point within the first 24 hours it is OK to continue the training. Over time you will be able to tolerate an increased load and see that the pain response decreases. On the other hand, if the pain response gradually increases as a result of the training load, you can adjust the load down to avoid a negative trend.

To summarise, we can say that pain is an important defence mechanism, but the need for protection depends on how we interpret the information at hand. Being confident in what the pain means for you, and how you should approach the situation, is important in order for you to start loading the body part in question.



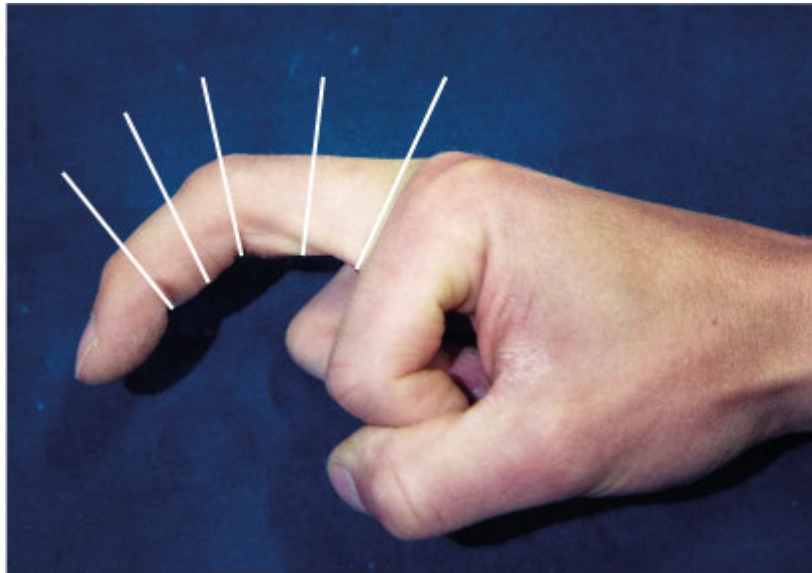
COMMON INJURIES AND AILMENTS

If we disregard injuries caused by falling – fractures, sprains and head trauma – we’re left with three distinct areas of the body where climbers tend to get injured. Finger injuries are the most common, followed by elbow and shoulder injuries. We have also chosen to include knees, as climbing-related knee injuries are becoming increasingly more common. Here we look at the climbing-related injuries and ailments we see and hear about most often, their causes, and what we can do when the damage is done.

FINGER INJURIES

PULLEY INJURIES

Finger pulley injuries are the most common climbing-related injuries. There are five pulleys in each finger – A1 to A5, from the base of the finger to the tip – with the exception of the thumb, which only has two. The pulleys are ligament reinforcements which lie on top of the tendons and work in the same way as the rings on a fishing rod, keeping the tendons in place along the bones and knuckles when the finger bends. When a finger is bending, the tendon pushes against the pulley, and if the load is high enough the pulley can rupture. This usually happens when you are gripping a hold and the load suddenly increases, perhaps because your foot slips or your other hand loses its grip. Crimping is closely related to pulley injuries because this grip position can lead to tremendous pressure being exerted on the pulleys, and this is one of the main reasons you should crimp with caution.



Finger injuries and painful fingers are unfortunately quite common in climbing, and it is mainly the pulleys, tendons and joints that are exposed.



Magnus Midtbø training with V-rings and a Progressor™.

When one or more pulleys rupture, either fully or partially, it's acute, and most climbers will experience a 'pop' in their finger when it happens. In many cases, the pop is also audible. You will experience pain emanating from the injured finger when you try to load it, and in many cases the injury will cause your finger to swell. It is recommended that you seek medical advice to correctly diagnose and assess the injury so that the rehabilitation can be as effective as possible.

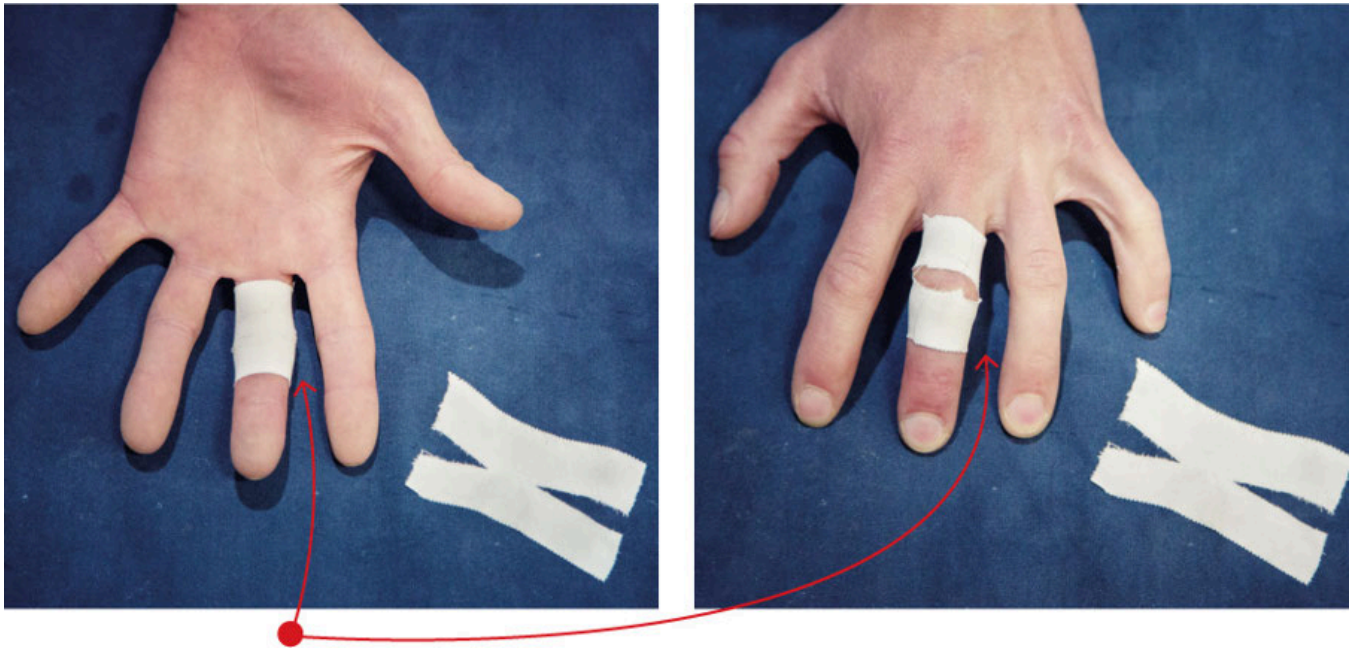
Full and partial ruptures of one or two pulleys are always treated conservatively, while surgery is considered an option if there is damage to three or more pulleys in the same finger. Depending on its extent, an injury is immediately followed by a period of one to two weeks of moving the finger, but with a reduced load. After the first phase is over, the laborious job of rehabilitation begins; this can take anywhere from four to twelve weeks before you can expect to climb at the same level as before. During this period, it is recommended that you use open hand grips and large holds when you climb, and control the load in relation to pain and swelling during and after training.

A fingerboard can be helpful since you can more easily control the grip position and load than when climbing. A handy tip is to stand on a bathroom scale and see how much of your own bodyweight you can lift before you feel

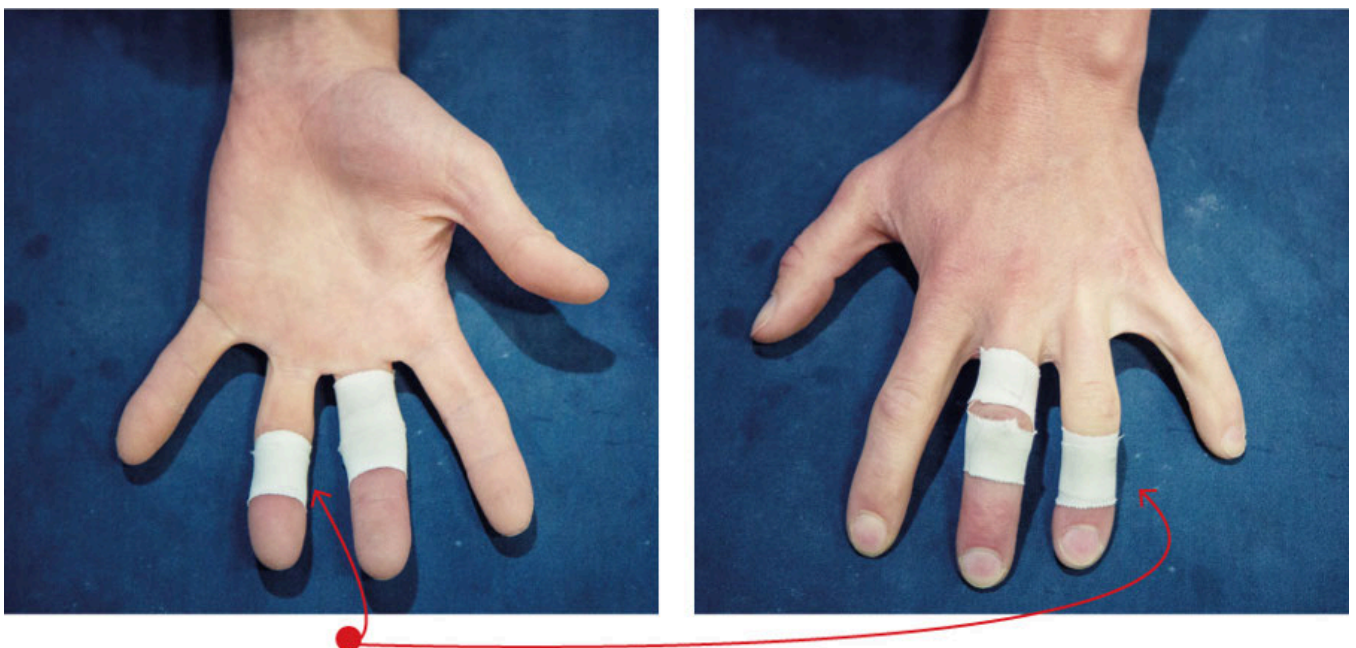
pain; this will give you a good indication of how far off you are from climbing at your normal level.

A better alternative is to use the Progressor™ mechanism from tindeq. This will allow you to measure the rate of force development for different grip positions, see how many kilos you can load before experiencing any symptoms in your finger and gradually increase the load as your finger gets better.

Even though torn pulleys do not grow back together, the prognosis is good, and most climbers recover to their previous level.



H-taping an A2 pulley injury.



Circular taping an A4 pulley injury.



It is recommended that you support the tendon by taping the finger for the first few months following a pulley injury. H-taping is recommended for injuries to the A2 pulley, and circular taping is recommended for injuries to the A4 pulley. The tape should be tight enough to provide support, but not so tight that the blood circulation is cut off. As your finger recovers and the load can be increased, the tape won't provide the same level of support, and there is no need for taping a previously injured finger when you are back to normal loading.

TENDON SHEATH INFLAMMATION

Tendons are encapsulated within the tendon sheath which produces synovial fluids so that the tendon can glide without friction against its surroundings. As the flexor tendons are pressed against the pulleys when you close your grip, the tendon sheath is squeezed in between, and by repeated high loading of the fingers the contact point between the tendon and the tendon sheath can become irritated, which leads to an increase of fluid in the tendon sheath. Applying direct pressure or loading the finger can be painful. The pain is often localised where there is a pulley, but unlike pulley injuries there isn't an acute event that leads to the injury. In most cases the injury is caused by too much loading of the finger in a short period of time, for example at the beginning of specific finger training or campusing, or when we start to climb more after a period away from climbing. The symptoms often last for several months even if we are good at adjusting the load and using more open grip positions. However, the prognosis is good, and it's important that you continue to climb, but avoid finger-heavy training like campusing, or bouldering and lead climbing on small holds.

FLEXOR TENDON STRAIN

Whereas injuries to finger pulleys and tendon sheaths are localised to one specific area in the finger, a strained flexor tendon will feel painful all the way from the finger up into the forearm where you find the connection between the tendon and the muscle tissue. The most common mechanism of injury is a sudden load increase while hanging off an open-handed one, two or three-finger pocket. Only the outermost joint will be bent, and if your foot slips in this grip position, the fibres connecting the tendon to the muscles can tear. It's often easy to test yourself after sustaining an injury like this: if you load the outermost joint using three fingers you will feel pain, but if you engage four fingers in a half crimp grip position there is no pain. This type of injury is seldom serious enough for you to abstain completely from climbing, but climbing with an open-handed grip position might be painful for several weeks after the injury. In this case, it is important to focus on easier climbing, primarily using four fingers in a half crimp grip position. As the pain starts to decrease during the first few weeks you can begin controlled hangs using open-handed grip positions on a fingerboard or at the climbing wall. It is however important that you focus on good foot placement so as to avoid any sudden and unexpected loading, and that you grip each hold while controlling the movement.

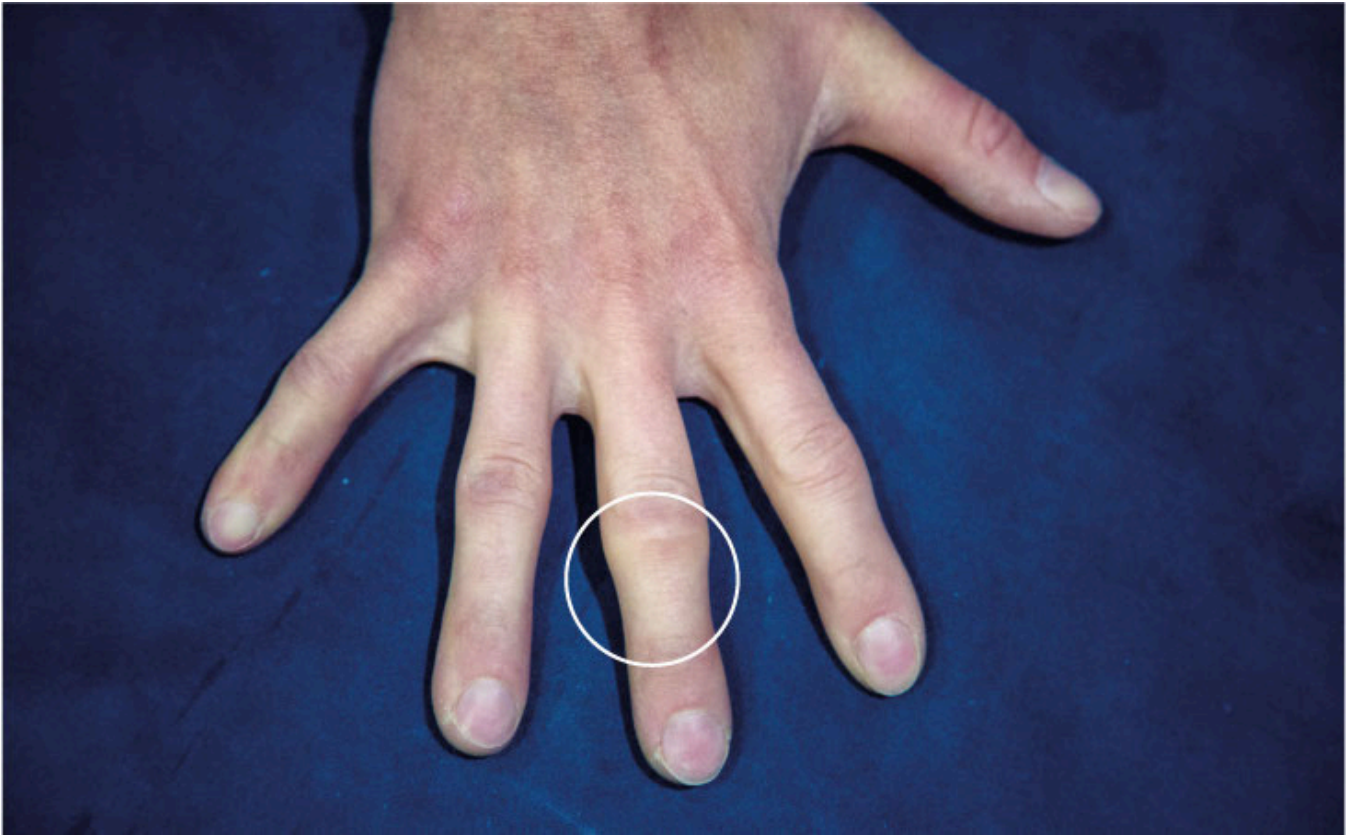
The extent of the injury can be assessed using ultrasound; if there is no sign of significant damage to the tissue, most people will be back to regular climbing after four to six weeks. More serious injuries can take longer to heal

and will require a more rigid approach; we recommend that you get a proper assessment before starting rehabilitation training.

By being aware of the risk involved when doing hard moves using open-handed grip positions, you can be more mindful of your foot placements and hitting the holds with as much control as possible to avoid acute strain injuries. The fact that finger injuries happen also when using open-handed grip positions lends weight to the argument that it is important to train climbing and deadhanging in different grip positions to be as strong as possible on all types of holds.



Flexor tendon strains usually happen when climbing on one, two or three-finger pockets.

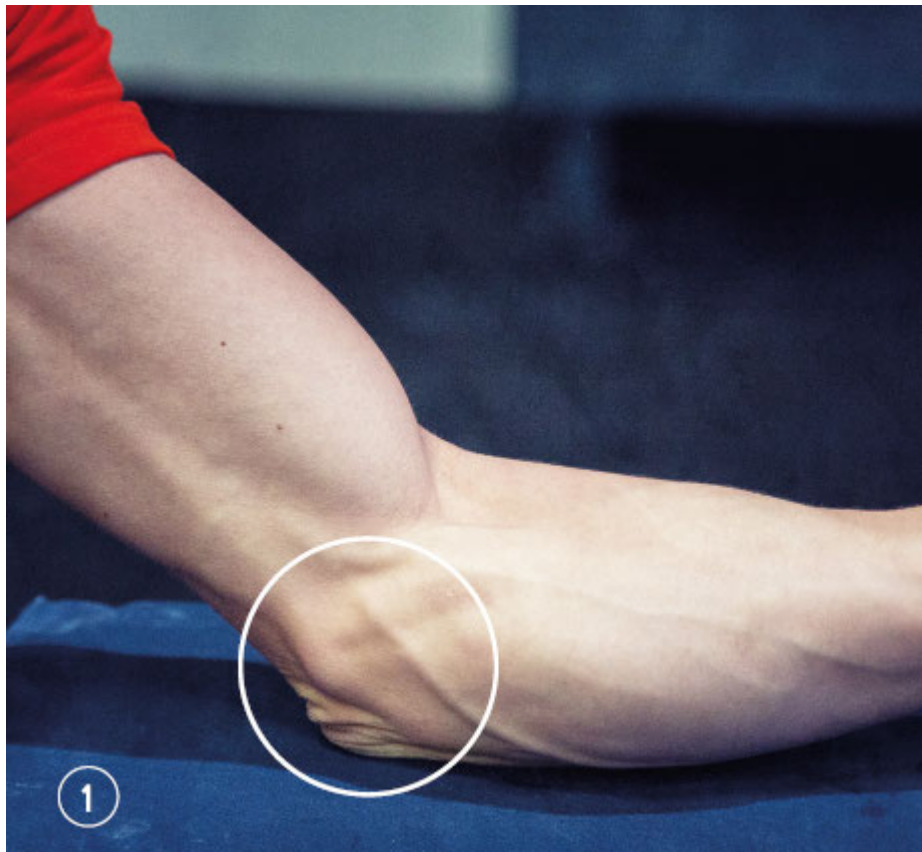


JOINT PAIN

In acute trauma, such as getting your finger stuck, or experiencing acute pain in the joint as you crimp, damage to the joint capsule and/or ligaments may have occurred. In such acute traumas, you will often experience swelling, and we recommend that these injuries are investigated by qualified medical personnel.

Far more common is diffuse pain in the joints that isn't caused by acute trauma. You might experience slight swelling, the finger can be tender to the touch and it can be difficult to bend the finger all the way into the palm of your hand. Mostly this is caused by too much heavy loading without enough time for recovery. For example, crimping places extreme stress on the middle and outermost joints of the fingers; this can lead to irritation in the joints, and more joint fluid being produced as a consequence. The joint will then swell and become tender to the touch and difficult to fully bend. This isn't dangerous but it should act as a hint for you to load your fingers less and focus more on easier climbing on better holds for a while.

While we know that the joints change with age and the number of years you have climbed, and that this is a natural consequence of loading your fingers heavily over several years, it has not been shown that these changes eventually lead to injuries that cannot heal. However, to avoid joint pain, we can assume that more care must be taken when crimping, among other things, as you get older. Even though it is not a serious injury, joint pain can still be detrimental to the quality of your climbing and prevent you from climbing as much as you want. This is yet another reminder for you to be conscious of varying your training and using as many grip positions as possible, giving you a varied load pattern on your fingers.



The flexor muscles of the fingers and wrist are connected to the medial epicondyle on the inside of the elbow.

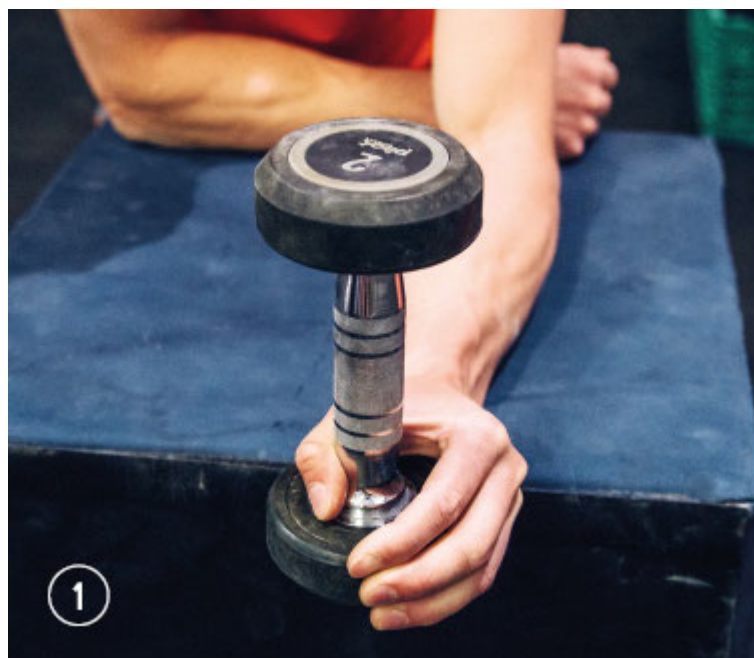


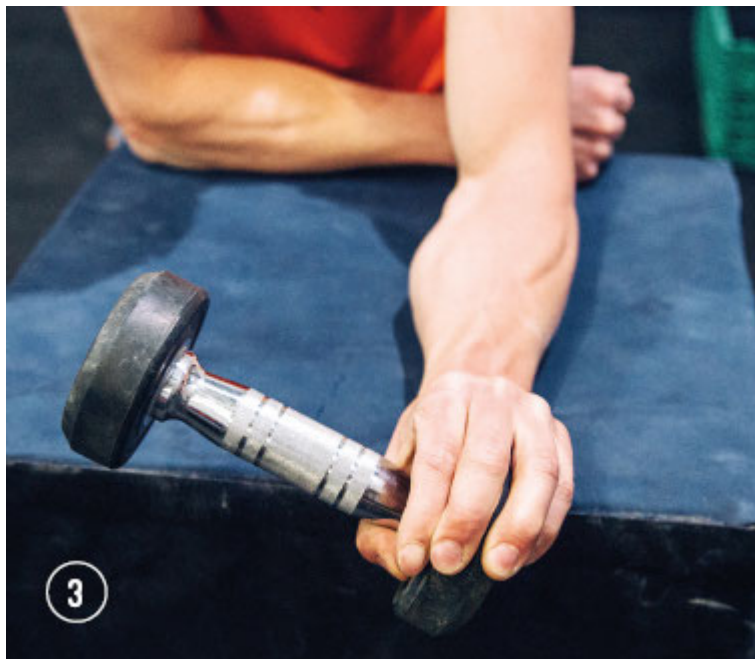
The extensor muscles of the fingers and wrist are connected to the lateral epicondyle on the outside of the elbow.

ELBOW PAIN

The muscles that bend and extend the wrist and fingers are connected to the inside (photo 1) and outside (photo 2) of your elbow, respectively – the medial and the lateral epicondyles. The musculature that rotates the forearm in and out is also connected to the epicondyles. Pain on the inside of the elbow is known as golfer's elbow, or medial epicondylitis, and is related to overloading of the connection between the flexor tendons and flexor muscles of the fingers and wrists, as well as the pronator teres muscle, which rotates the forearm so that the palm faces away from us. Pain on the outside of the elbow is known as tennis elbow, or lateral epicondylitis, and is related to overloading of the connection between the extensor muscles of the fingers and wrists, as well as the supinator muscle, which, together with the biceps brachii, rotates the forearm so that the palm is facing towards us.

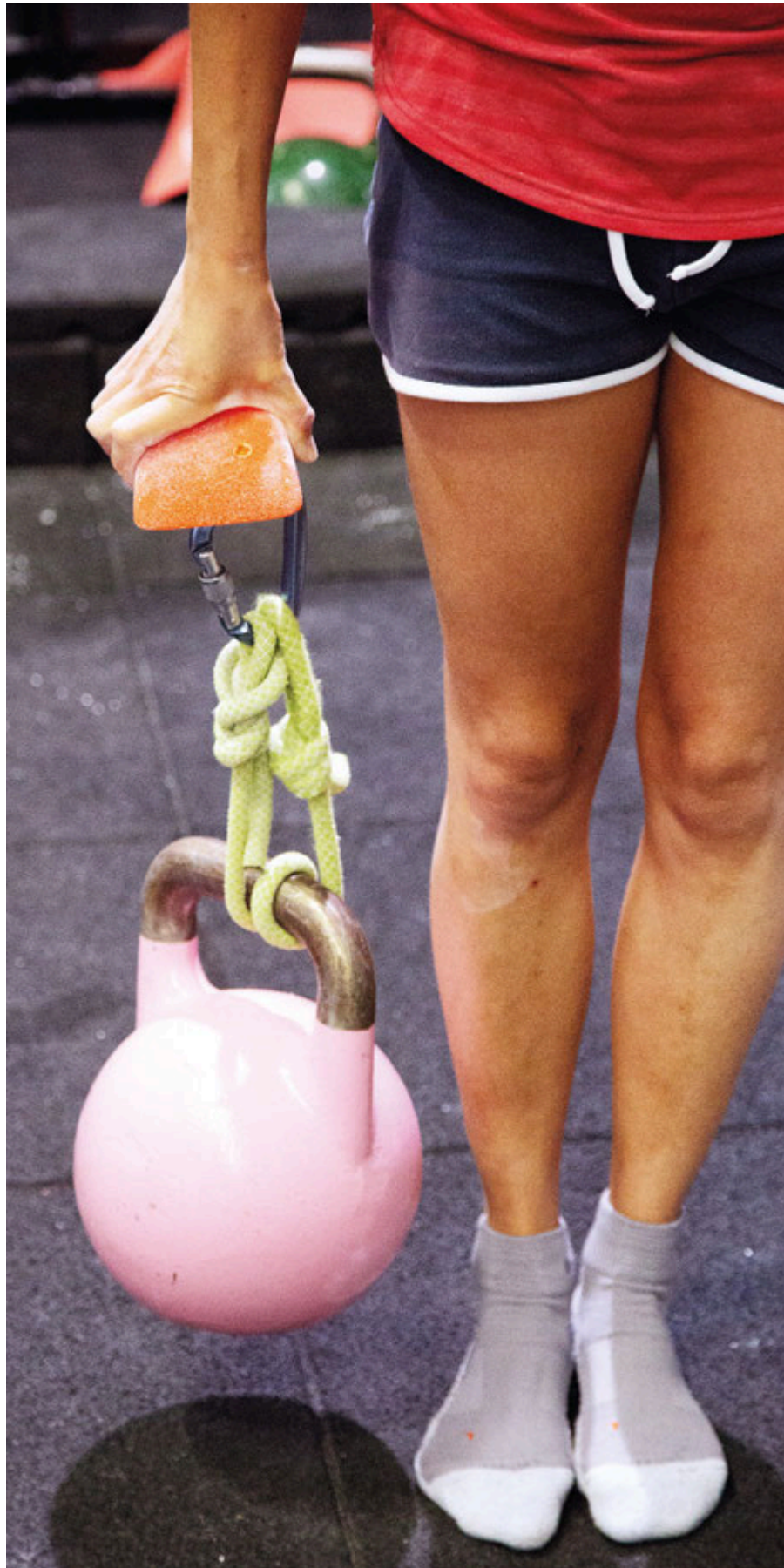
All of this musculature contributes when we are gripping holds, and the pain is usually caused by too much and too hard training without sufficient rest. The most important counter measure is load management, as described earlier in the chapter. For example, switching from bouldering to lead climbing to lower the intensity of the training, or climbing on walls that are less steep so that more weight is transferred from the feet to the arms and fingers than on steeper climbing. Since the pain usually stems from the connection between the muscles and the tendons, it is important to be patient during rehabilitation since tendons are notoriously slow to adapt to changes in load. A three to twelve-month time perspective is not uncommon and underlines the importance of finding ways to climb that do not provoke an excessive pain response during and after climbing. Here, a training log with pain monitoring, as described earlier, is useful for properly managing the training load. Once the load management is in place, it is advisable to start with one or two specific strength training exercises to stimulate the tendon to adapt. These exercises appear to be most effective when exercising hard, which means that you should be able to perform a set but you should feel tired for the last two repetitions – so choose weights accordingly. Some possible exercises can be found on the following pages.





1. FOREARM ROTATION

Grip the dumbbell and rotate it through 180 degrees so that your palm is facing up towards the ceiling and then down towards the floor. Allow 3 to 4 seconds for each movement for a total of 6 to 8 seconds per repetition. Complete three sets of six to eight repetitions, three times a week.



2. PINCH

Thread a rope through the bolt hole of a pinch hold and attach weights to the rope. Lift the weights off the ground by pinching the hold and bending your elbow. Allow 3 to 4 seconds on the way up, and 3 to 4 seconds on the way down, for a total of 6 to 8 seconds per repetition. Complete three sets of six to eight repetitions, three times a week.





3. WRIST CURL

Rest your forearm on a table or similar with your hand hanging off the edge and your palm facing down. Lift a dumbbell up and back. Allow 3 to 4 seconds on the way up and 3 to 4 seconds on the way down, for a total of 6 to 8 seconds per repetition. Complete three sets of six to eight repetitions, three times a week.

Exercises 1 and 2 are recommended for medial elbow pain, and exercises 2 and 3 are recommended for lateral elbow pain.

There is also a third source of elbow pain, known as climber's elbow. The pain is localised to the centre of the front of the elbow, approximately where you can feel the bicep tendon, and can be provoked by loading and bending the elbow. The pain can be related to the bicep tendon, but in most cases it is caused by overloading the brachialis muscle, which lies underneath the bicep muscle. While the bicep is dependent on the forearm being rotated so that the palm is facing up – imagine holding an undercling – the brachialis is not. This means that when you bend your elbow with the forearm rotated such that the palm is facing down – imagine doing an overhand pull-up – the brachialis has to work harder because the bicep is not in a position to exert maximum force. The pain is usually a consequence of too much training involving pull-ups, campusing and steep bouldering. Again: when pain is a result of overloading, the best countermeasure is to improve load management. First and foremost, you must identify which training methods are provoking the symptoms so that you can avoid these for a period and gain control of the situation. The easiest measures are to avoid specific strength training of the elbow flexors and do less climbing on steep walls. It's often better to compensate by doing more and easier moves instead of fewer and harder moves, and as the symptoms start to fade you can gradually climb steeper and harder.

FONT ELBOW

Climber's elbow is also known as 'Font elbow'. Most of the boulders you will find in the bouldering mecca of Fontainebleau in France have a rounded top, often with nothing more than pure friction as the only means of

topping out. You must then press the palms of your hands down on the holds at the same time as bending your elbows to pull your body up. This movement pattern is central to almost all of the top-outs, independent of the level of difficulty of the boulder problem, and is repeated every day you are out climbing in Font. Many climbers will end up suffering from climber's elbow when visiting Fontainebleau, hence the name Font elbow.

SHOULDER PAIN

The shoulder has the largest range of motion of all the joints in our bodies, and we use our shoulders in a variety of ways every time we climb. Because the joint is so mobile, by nature it is relatively unstable, and great demand is placed on muscular strength and control to maintain stability of the shoulder while it is in motion. There are several possible causes of shoulder pain, and even the experts sometimes disagree on how to classify and diagnose shoulder pain. We therefore recommend seeking professional medical help if you experience shoulder pain which affects you in your climbing. However, some basic guidelines may be useful if your shoulder is painful.

With the exception of major injuries such as shoulder dislocations, fall trauma and major tendon ruptures, most shoulder injuries in climbing occur due to overloading of one or more structures adjacent to the shoulder. The structures that are most frequently affected are the synovial bursae and rotator cuff tendons with their associated muscles. The rotator cuff consists of four muscles – supraspinatus, infraspinatus, subscapularis and teres minor. Collectively, these form a cuff around the shoulder joint which helps to stabilise the ball on the upper arm in the joint socket, and around this cuff lie a series of synovial bursae which ensure that the movements between the tendons and the surrounding tissue are as frictionless as possible. In simple terms, all the climbing moves we do expose the rotator cuff and bursae to loading and compression, and excessive strain over time can cause discomfort in these structures. Pain in some arm positions and during some climbing moves is a common characteristic, as is pain when lifting the arm above shoulder height and with everyday functions such as getting dressed or putting the seat belt on in the car.

Since shoulder pain in climbing is associated with overloading, it is important to adjust the load by switching exercise methods and reducing the intensity of workouts – remember Greg Lehman's quote about calming down and then building back up. After you have regained control of your training load, shoulder pain is treated primarily through various training exercises. Which exercises and how hard they should be is determined in consultation with medical professionals who are experienced in dealing with shoulder pain, and preferably someone who also has climbing experience, so that rehabilitation can be directed towards the movement pattern you are returning to. If shoulder pain is a recurring problem, we also recommend that you seek guidance from a coach who can help you identify various movement patterns and technical solutions you use that may be playing a part in causing the discomfort. You can then use climbing-specific strength and technique exercises for both rehabilitation and prevention of future injuries.

Since the shoulder is primarily stabilised by muscles, it is important to build a good physical foundation of strength surrounding it so that it can gradually endure more loading as your climbing progresses. Earlier in the chapter, we presented different exercises to help with this, and which also help to create a balance in the strength relationship between the primary movers and their antagonists.

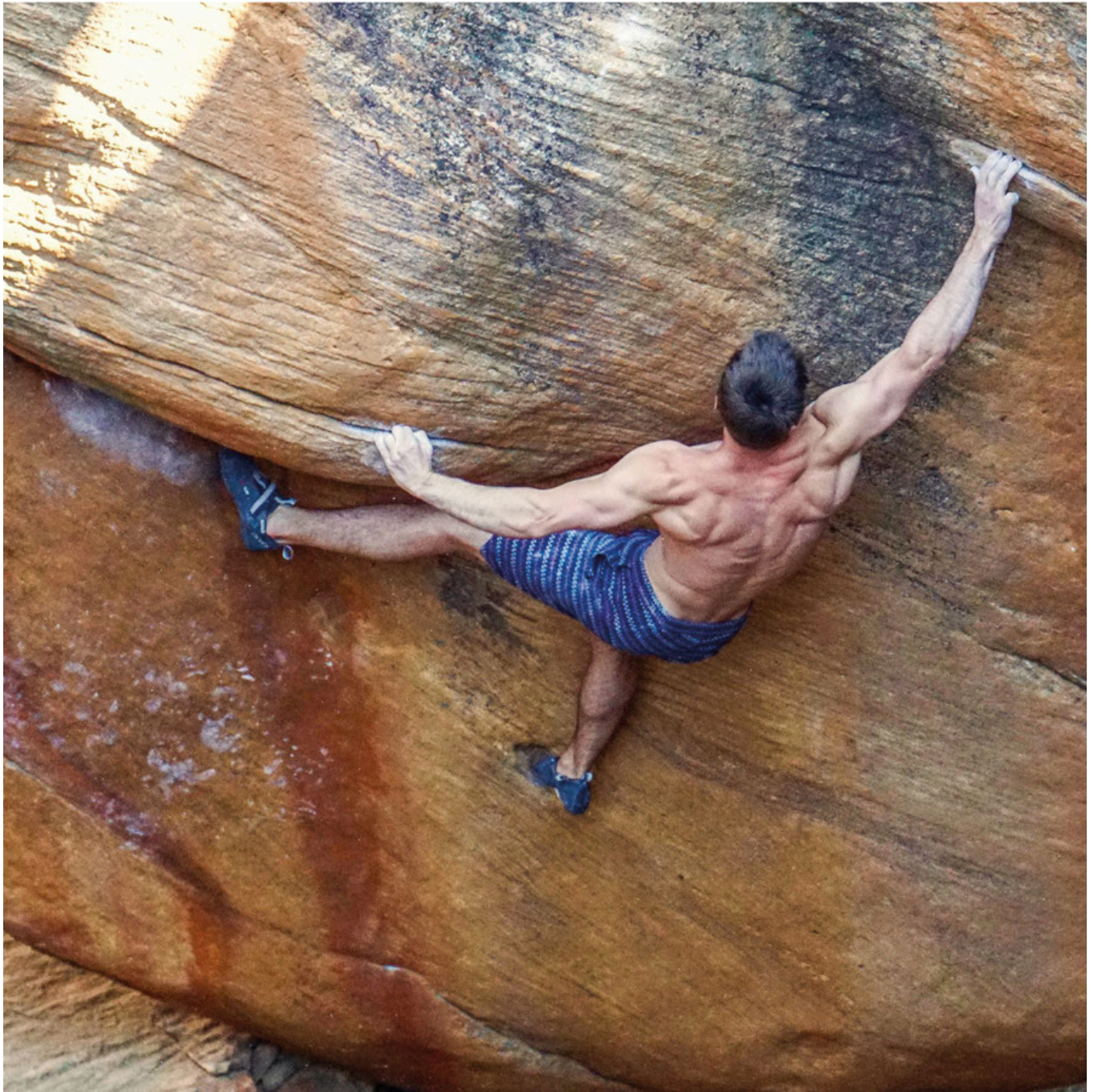
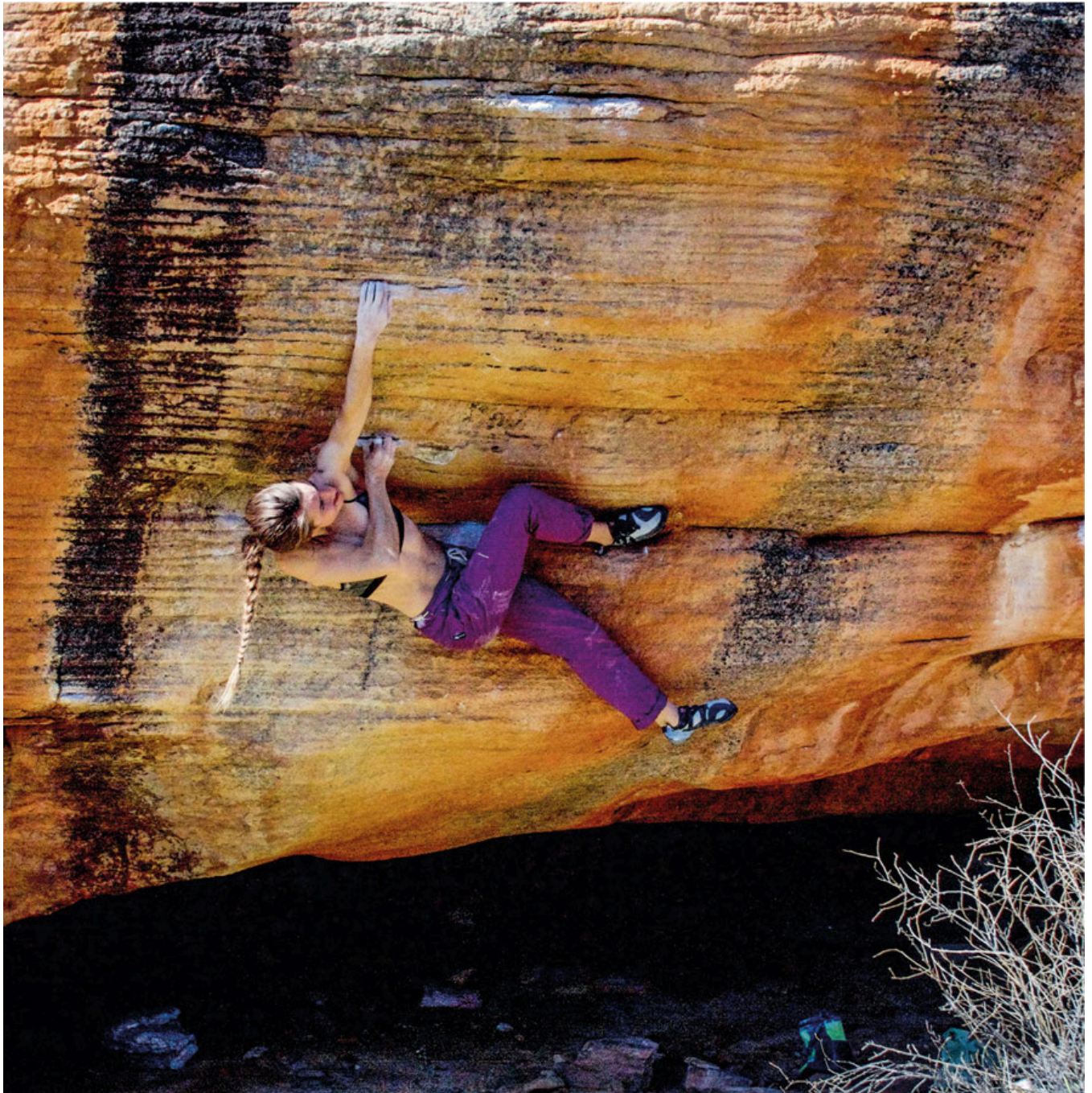


PHOTO: KATRINE VANDET SALLING

Thilo Schrøter climbing El Corazon (Font 8b), Rocklands, South Africa.



Mari Augusta Salvesen climbing Pinotage (Font 7b+), Rocklands, South Africa.



KNEE INJURIES

The knee joint is basically a stable joint, and it takes large forces to damage the various structures around it. Most climbing-related knee injuries are therefore associated with acute events on the wall or falls. When we place the knee in extreme positions such as heel hooks, twisting in and rockovers, the knee joint, the cruciate ligament and the collateral ligaments are subjected to great strain, and experience shows that most acute, climbing-related knee injuries occur in these positions.

As with all acute injuries we recommend that acute knee injuries are examined by a medical professional who can assess the damage. Severe injuries might require surgery, but most injuries involve only minor damage to ligaments and menisci, and rehabilitation focuses on building strength around the knee while the injured knee heals. It is therefore important to have the injury examined in order to choose the right path for rehabilitation.

In the selection of exercises presented earlier in the chapter, we have deliberately added strength training exercises that target the musculature around the knees. In several other sports, it has been shown that strength training can reduce the risk of acute knee injuries, and even though these injuries aren't especially common in climbing, it is a good idea to also include some strength training for the legs. Still, what is most important is to look at which positions on the wall are likely to put your knees in harm's way and then try to reduce the number of times you do these moves when training. Obviously, you will do anything within your power to get to the top when redpointing, which will include heel hooking, twisting in and rocking over, but you can reduce the risk of knee injuries if you limit these techniques in your day-to-day climbing training – or at least be aware of the risks involved when using them. And, if you feel the need to use such techniques, make sure you warm up your legs just as you would your fingers, for example, by using the exercises described in this chapter.



PHOTO: BJØRN HELGE RØNNING

Teodor Kirkebø in the 2017 Junior National Bouldering Championship finals in Oslo.

YOUNG CLIMBERS AND THEIR RISK OF INJURY

We have chosen to define young climbers as those below the age of 16, and the most important message to this group is that until they are fully grown up they must take special care of their fingers. While the bones in the body are growing, youths are particularly prone to injury in the growth plates. The growth plate – known as the epiphysis – is a disc near the end of a bone that is softer than the rest of the bone tissue. The epiphysis allows the bone to grow and become longer, and it closes and hardens when the body is fully grown, which for some people will not happen until the age of 18. Until it has closed it is especially prone to injury. The epiphyseal plates of the fingers lie in connection at the joints in the fingers, and the middle joint in particular seems to be prone to injury when exposed to too much finger-intensive training and climbing. This is the reason why we do not recommend deadhangs and campus training for young, growing athletes, as specific finger training at this age can lead to long breaks from training and, in the worst cases, injuries that might never heal. Care should also be taken during finger-intensive climbing, and crimping should be reduced to a minimum.

In cases of pain in the fingers, such as pain before, during and after training, and swelling in the middle joints of the fingers, it is advisable to contact a qualified medical professional who can assess whether there is damage to the epiphyseal disc. Finger discomforts such as these should always lead to a reduction in training load, but if the injury can be detected through MRI or ultrasound examination, a two-month break from climbing is recommended before re-examination of the finger. If you take the symptoms seriously, your fingers will finish growing and the injury will not present any problems in the future, but if you let the condition go too far, your fingers can be afflicted with permanent, non-healing injuries, which may end your climbing career.

During puberty, both body height and body mass will increase, and during this period we must be aware of the risk of injury since an increase in body mass leads to increased body weight, and thus a higher load on the fingers. This period is estimated to be from the age of 11 to 14 for girls, and from 12 to 16 for boys. This often coincides with a time when many ambitious young climbers are gradually adding more and harder training, and this combination is not without its problems. It is important that the athletes themselves, and we as adults, are aware of this issue and organise the training in a way that does not overload the fingers. This involves using larger holds and open grip positions, avoiding specific finger training like deadhangs and campusing, and having a continued focus on versatile and varied climbing training.

For young and ambitious climbers, the increase in body weight during puberty can be difficult to deal with. Many will face a period of reduced performance and perhaps see their peers develop faster than they are doing. During this period it can be tempting to increase performance by reducing weight, and it is important to be aware of the risks involved. Reduced weight equals energy deficit. This means you are supplying your body with less energy than it consumes, and this energy deficit can have consequences for your health. Women have what is known as the female athlete triad, which describes the interrelationship of a negative energy balance, menstrual dysfunction and decreased bone mineral density. The combination of these three components can lead to permanent health damage. In some athletes, focusing too much on food and weight can lead to a disrupted eating behaviour or an eating disorder. This is a mental illness that can lead to both physical and mental health complications, and is a potentially lethal affliction. This can also happen to male athletes, and even though there is no such thing as the male athlete triad, their problems are related. Using weight reduction as a performance enhancer at a young age can lead to serious physical and mental illness forcing an early exit from climbing. In addition, it is impossible to get enough quality training over time with a constant energy deficit. Eventually, you will drastically deteriorate physically, and it is often challenging to train yourself back to your previous level. Many will lack the motivation needed to put in the effort and will end up quitting instead. It is extremely important to nurture a healthy training environment for youth climbers to develop at their own tempo, where nutrition and puberty are natural topics to talk about so that they aren't driven towards a premature focus on weight loss as a performance-enhancing tool.

These are our recommendations for reducing the risk of injury in younger climbers:

- Training should be varied and fun.
- Focus mainly on doing more moves rather than harder moves.
- Spend most of the training time learning technical skills and developing a good movement pattern on the wall.
- Train the arms and upper body, but avoid specific finger strength training. Focus on technical execution, simple training methods and slow and steady progression.
- Use general strength training for the whole body, as well as training balance, coordination and endurance.
- Be aware of symptoms in the fingers throughout puberty, but especially during periods of rapid increase in height and body mass.

A CLIMBER'S STORY

Norwegian climber Rannveig Aamodt was born in 1984. She is a trained veterinary animal therapist and has a background as a teacher at a high school. She now lives in Colorado as a professional climber and photographer. On 26 April 2012 she experienced a near-fatal climbing accident.



A CLIMBER'S WORST NIGHTMARE

Life is full of moments that change us forever, and 26 April was one of them. That day I fell to the ground and broke my back, pelvis, one arm and both my ankles. My life changed forever.





IT WAS SPRINGTIME and we were living the life in Geyikbayiri in Turkey. My husband Nathan and I were eating breakfast and drinking coffee while flipping through the guidebook and making plans for the day, as we always did. We ended up at a crag called Sarkit and started warming up as usual.

After Nathan had led a steep, athletic route, I followed on top-rope. Because another crew also wanted to top-rope the route, I brought their rope up with me, and as I unclipped our rope I clipped theirs in. I climbed easily, dancing between small footholds and large tufa. At the anchor I first clipped myself in before clipping in their rope, but because it had gotten twisted and tangled, I had to unclip their rope, untangle it and clip it back in.

And this is when it happened. A quick slip of the mind, shaped by old habits and a sense of security. I looked down at the rope clipped into every quickdraw and the anchor at the top, a common and ordinary sight. It was just that this time I wasn't on lead, and Nathan wasn't holding the other end of that rope. So when I clipped the other rope into the anchor, I unclipped my own rope, the one Nathan was holding. 'You got me?' I called down. Nathan leant back, felt the rope come tight because I was still clipped to the anchor and shouted back. I unclipped myself from the anchor – and let go.

FROM NATHAN'S DIARY:

I can still picture her falling. In a seated position, her arms making small circles in the air. Like a bird falling out of her nest. I can still feel my gut wrenching for every metre of rope that doesn't take. She made the same sound as when she accidentally dropped a plate on the kitchen floor, and I can hear the sickening sound of bones crushing and flesh tearing. The echo of my voice as I screamed for help at the empty fields below us.





I woke up in a bubble of light, confused, but vaguely aware of what had just happened. I remember screaming and drifting in and out of consciousness. I remember moments of being awake with extreme pain in my feet, legs and back, and this went on for hours until I arrived at the hospital.

I woke up after the operation and looked down at my mummified body. I was just able to move my toes, to the great relief of those around me who feared I was paralysed. I had three compression fractures in my back, a broken pelvis, two fractured ankles, and several broken bones in my feet. I had shattered my right elbow and partially torn my tricep tendon off.

I was a healthy and strong athlete who had spent the last few years of my life climbing all over the world, and here I was: trapped in a body that didn't work and in tremendous pain. I couldn't go to the bathroom on my own, I couldn't wash myself, I couldn't even turn over in bed. Yet still, I felt enormous gratitude overwhelming everything. All the small things I had previously taken for granted were like receiving gifts – from getting my back scratched to getting my hair washed. It was like every emotion was intensified, and I was on a journey to my inner self.



‘AFTER ABOUT TEN WEEKS I COULD START TO PUT SOME WEIGHT ON MY FEET.’

CLIMBING BACK TO LIFE

I had a lot of time with my thoughts. Will I be able to climb again? And, if so, will I be able to get back to my previous level? Will I even want to climb again?

I decided that climbing should be something I do for myself because it's something I love doing. Climbing was to be a tool of encouragement, helping me fulfil my potential and pushing me to be at my best, independent of what level I was currently climbing at. And then I had to make a distinction between who I am and what I do. Climbing is something I do. I am not a climber. Whatever I might lose, I'm still me.

Amid all the fear and anxiety this gave me a sense of clarity and calmness that let me accept my situation, and I started to think positively about what was ahead of me. It triggered a force within me, built on defiance and determination to embark on the journey ahead.

I started lifting a weight with my left arm, the one which was still working. I underwent multiple surgeries in Norway and spent weeks in the hospital and rehabilitation facilities while I continued training as much as I could. To begin with, I sat in a wheelchair and could only stretch my arms and legs and train using slings. My brother brought me kneepads so that I could start crawling on the floor, and I ended up doing alarming amounts of crawling.

After about ten weeks I could start to put some weight on my feet. I started hanging from a harness over a treadmill and progressed by lifting weights, cycling on an exercise bike and swimming. Every step forward built on what I had previously done, and the feeling I got from seeing the results of all the hard work that I had put in was extremely motivating. My biggest challenge was the pain. Previously I had used pain as an indicator for when I pushed too hard during training, but since the pain was now constant, this indicator was useless. I had to push through the pain in order to progress, but I also had to know when to rest. I learned that pain creates fear, and if you give in to this fear it limits everything.



From knowing my own limits I had to jump into the deep end and learn how to swim again, and frequently I realised that my limits were much further out to sea than I had imagined.

When I finally emerged from the rehabilitation centre I started working with Stian. It marked the end of seeing myself as a fragile patient, and together we changed my focus towards dedicated training to become a better climber. I had my dreams and goals, and he helped me find ways to achieve them.

Eight months after the accident I went to Thailand, and it was then that all of my training manifested itself into climbing performance. Eight months to the day after the accident, I sent my first F8a, and I ended the trip with an additional two F8a+, two F8a, and a F7c. Now, six years after my accident, I have been back to the route where I almost died so I could look death in the eye, I climb F8b+ and I'm living as a professional climber.

However, the constant pain is exhausting to this day. Sometimes it feels like climbing and hiking takes more than it gives, which makes me question what I'm really doing. Then I have to remind myself that I am grateful. I am grateful to be able to do what I do, and just as the pain in my body after a long day in the mountains used to make me happy, I try to look at it in the same way now – to relate the pain to something satisfying and a result of having achieved something.

It has been a long journey, and it is far from over, but it has strengthened my belief that in order to realise your dreams you need to give it 100 per cent. That your criteria for success are your own. That your goals and dreams are your own and not anybody else's. Dream big and work hard to get there, because no matter what, it will lead you somewhere. Not always where you had in mind or wanted to go, but often it will take you much further than you could ever have imagined.



‘DREAM BIG AND WORK HARD TO GET THERE, BECAUSE NO MATTER WHAT IT WILL LEAD YOU SOMEWHERE.’
